

How to Open a Behavioral Health Residential Facility in Arizona

A step-by-step guide to licensing and opening a BHRF — 24-hour residential treatment for mental health and substance use — and getting paid by AHCCCS

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What a BHRF is — read this first

A **Behavioral Health Residential Facility (BHRF)** is a licensed, 24-hour residential treatment setting for people with a mental health or substance use condition serious enough to need round-the-clock supervision and support while they are in treatment. It is not a sober living home (that is housing without treatment, licensed separately) and not an outpatient clinic. A BHRF is a health care institution licensed by the Arizona Department of Health Services under Article 7, and it is paid by AHCCCS (Arizona Medicaid) as a behavioral health level of care. Opening one means clearing two big gates: the ADHS license and the AHCCCS contract. This guide walks you through both, plus the one financial fact that surprises every new operator — the AHCCCS rate does not include room and board.

How to Use This Guide

This guide takes you from idea to a licensed, AHCCCS-contracted BHRF. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Arizona operators — not legal, clinical, or billing advice. BHRF rules are detailed and are actively updated by ADHS and AHCCCS, so verify each step with the agency named before you rely on it. Where this guide gives a code, a fee, or a figure, treat it as a starting point to confirm.

The one-paragraph version

Form an Arizona business entity and secure a property that can pass the physical-plant and fire requirements for a residential health care institution. Apply to ADHS for a Behavioral Health Residential Facility license under Article 7 — decide your capacity (a clinical director is required at 10 or more beds) and pass the fire inspection and licensing survey. Build your clinical team: an administrator, a Behavioral Health Professional (BHP) for clinical oversight, and behavioral health technicians, all with Fingerprint Clearance Cards. Enroll with AHCCCS as a provider (Provider Type B8) and contract with the AHCCCS Complete Care health plans. Understand how you get paid: AHCCCS pays a per-diem for the treatment (billed as H0018) but not for room and board, which you cover another way. Members reach you by referral from outpatient teams, crisis services, hospitals, and courts, with prior authorization. Then run the facility to the treatment-plan and documentation standards AHCCCS and ADHS require.

The Path at a Glance

Ten steps take you from idea to an operating Arizona BHRF:

1. **Decide your model.** Population (mental health, substance use, or both), capacity, and whether you will serve AHCCCS members (almost always yes).
2. **Form the business.** Register your entity with the Arizona Corporation Commission and get an EIN.
3. **Secure the property.** A home or building that can meet the residential physical-plant and fire-safety requirements for your capacity.

4. **Write your policies.** Adopt the BHRF policy and procedure manual, forms, and clinical documentation the license requires.
5. **Build your team.** Administrator, a Behavioral Health Professional for oversight, and behavioral health technicians — all fingerprint-cleared.
6. **Apply to ADHS.** Submit the initial Behavioral Health Facility License application under Article 7 with the fee, drawings, and fire inspection.
7. **Pass the survey.** ADHS reviews your application and inspects the facility before issuing the license.
8. **Enroll with AHCCCS.** Register as an AHCCCS provider (Type B8) through APEP and contract with the AHCCCS Complete Care health plans.
9. **Solve room and board.** Plan how you will cover the room-and-board cost the AHCCCS per-diem does not pay.
10. **Open and take referrals.** Accept authorized admissions from outpatient teams, crisis services, hospitals, and courts; treat, document, and discharge to plan.

Part 1 — What This Business Is

A Behavioral Health Residential Facility provides 24-hour treatment, supervision, and support to adults (or, in some subclasses, youth) whose mental health or substance use condition is serious enough that they cannot yet live safely on their own, but who do not need a hospital. Residents live at the facility while they participate in structured treatment — counseling, skills building, symptom management, substance use treatment, medication support, and daily structure — with the goal of stabilizing and stepping down to a lower level of care in the community. In Arizona’s system, a BHRF is a defined level of care that a member is placed into by their treatment team when they meet medical-necessity criteria.

This is a genuine treatment business, and it fills a real and growing need. Arizona’s behavioral health and substance use demand is high, the state has invested heavily in residential capacity, and AHCCCS funds this level of care. But it is also a licensed, clinically supervised, audited setting — much closer to running a small treatment program than to running a group home. Go in understanding that you are opening a health care institution, not just a house.

What a BHRF is not

- **Not a sober living home.** A sober living home provides substance-free housing and peer support but no treatment, and is licensed separately under a different Arizona law. A BHRF provides actual behavioral health treatment and is licensed as a health care institution.
- **Not an outpatient clinic.** An Outpatient Treatment Center (Article 10) delivers behavioral health services without residents living there. A BHRF is 24-hour and residential.
- **Not a psychiatric hospital.** A BHRF is a non-hospital residential level of care — below inpatient, above outpatient.

Subclasses to know

Beyond the standard BHRF, Arizona licenses a Secured BHRF (locked, court-ordered placements under A.R.S. § 36-425.06), an Adult Behavioral Health Therapeutic Home (a smaller family-style setting), and a recidivism-reduction subclass for adults leaving the justice system. Most new operators open a standard BHRF; the others carry extra requirements. Decide which one you are before you apply.

Why Arizona, and why now

Arizona has leaned hard into community behavioral health. The state runs a robust crisis system, AHCCCS funds a full continuum of behavioral health levels of care, and residential capacity is in genuine demand — for adults stepping down from psychiatric hospitalization, for people stabilizing from a substance use crisis, and for members the courts and crisis system need to place quickly. Because AHCCCS pays for this level of care and members reach it through an organized referral system, a well-run BHRF that keeps its beds authorized and its documentation clean can stay full. The need is real and, in most of the state, under-served.

Be honest with yourself about what you are taking on, though. A BHRF is a clinical, regulated, audited business. You are responsible for people in acute need, around the clock, and both ADHS and AHCCCS will hold you to it. The operators who succeed come in with clinical seriousness, adequate capital to survive the months before revenue starts, and a plan for the room-and-board gap. The ones who struggle treat it like a real-estate play and discover, too late, that the license, the staffing, and the documentation are the actual business.

Substance use, mental health, or both

A BHRF can serve members with mental health conditions, substance use conditions, or both, and Arizona’s rules and AHCCCS’s medical-necessity criteria cover the full range. Your choice shapes your clinical model: a substance-use-focused BHRF works within the ASAM levels of care and lives with Certification-of-Need and MAT requirements; a mental-health-focused BHRF centers on psychiatric stabilization, symptom management, and skills. Many facilities serve co-occurring conditions, which is common and often expected. Decide your population

early, because it drives your staffing, your BHP's expertise, your admission criteria, and the plans you contract with.

Part 2 — The Two Gates: the ADHS License and the AHCCCS Contract

Almost everything about opening a BHRF comes down to two approvals, and you need both. The first is the license from the Arizona Department of Health Services — without it you cannot legally operate a residential health care institution. The second is the provider agreement with AHCCCS and its health plans — without it you have a licensed building but no way to get paid, because nearly every BHRF resident is an AHCCCS member whose stay is authorized and funded through the behavioral health system.

New operators sometimes treat these as one step or assume the license is the whole job. It is not. You can hold an ADHS license and still have no revenue if you are not enrolled and contracted with AHCCCS, and you can be AHCCCS-enrolled but unable to admit anyone without the license. Plan and budget for both tracks in parallel, and expect the whole path — property, license, and contract — to take months, not weeks.

The mindset that keeps a BHRF open

Run it like the health care institution it is. A BHRF lives or dies on clinical documentation, medical-necessity, and authorization. Every resident needs an assessment, a treatment plan, ongoing BHP oversight, and continued-stay justification. The operators who thrive treat paperwork and clinical rigor as the business — because AHCCCS pays for documented, authorized, medically necessary care, and audits everything else back.

Part 3 — Getting Licensed by ADHS (Article 7)

The Arizona Department of Health Services licenses BHRFs as behavioral health residential facilities under Arizona Administrative Code, Title 9, Chapter 10, Article 7. You apply on the initial Behavioral Health Facility License application, and ADHS reviews your application and inspects your facility before issuing a license.

What the application involves

- **The application and fee.** The initial application carries a non-refundable application fee (recently \$50), plus licensing fees set by rule and scaled to your capacity. Fees are non-refundable, so submit a complete, correct packet.
- **Your capacity.** You choose your licensed resident capacity. This is a pivotal decision: at a licensed capacity of 10 or more residents, you must designate a clinical director, and larger capacity raises your physical-plant and staffing requirements. Many new operators start smaller and grow.
- **The physical plant.** The building must meet the residential health-care-institution standards for bedrooms, bathrooms, living and treatment space, and safety. If you construct or modify space, you submit architectural drawings signed by an architect that meet the adopted building and fire codes.
- **The fire inspection.** You submit a current fire inspection from the local fire department. Fire and life-safety compliance is one of the most common places a residential application stalls — handle it early.
- **Ownership and controlling persons.** ADHS reviews the applicant and controlling persons; a disqualifying history can bar a license.

After a complete application, ADHS runs an administrative-completeness review and a substantive review within the timeframes published in its rules, inspects the facility, and then approves or denies the license. Build the review and inspection time into your opening plan, and do not sign residents or bill before the license is issued.

A few practical realities make the licensing track go faster. Submit a genuinely complete packet the first time — missing fire inspections, unsigned drawings, or gaps in your policies are the usual causes of delay, and because the fees are non-refundable, a rejected packet is expensive as well as slow. Have your policy and procedure manual, clinical documentation forms, and staffing plan ready before you apply, because ADHS wants to see that the facility will actually operate to the rules, not just occupy a building. And keep in mind that the license names your capacity, your subclass, and your controlling persons — changing any of those later is its own process, so decide them deliberately up front.

Decide your capacity deliberately

The 10-bed line matters. At 10 or more residents you must have a clinical director in addition to your administrator and BHP, and your physical-plant and staffing obligations grow. A smaller first facility is cheaper to license, faster to staff, and easier to run well — and you can scale once you have a track record. Pick capacity based on the team and building you can actually resource, not the maximum you can imagine filling.

Part 4 — Building Your Clinical Team

A BHRF is only as strong as the people in it, and Arizona sets specific roles you must fill. Recruit these before you apply, because ADHS wants to see that the facility will be properly staffed and supervised.

- **Administrator.** The person responsible for the day-to-day operation of the facility, meeting the qualifications in the rules. Every BHRF has one.
- **Clinical Director (10+ beds).** If your licensed capacity is 10 or more residents, you must designate a clinical director who oversees clinical services and meets the professional qualifications in the rules.
- **Behavioral Health Professional (BHP).** A licensed professional — for example a licensed independent substance abuse counselor, licensed professional counselor, licensed clinical social worker, licensed marriage

and family therapist, psychologist, or psychiatrist — who provides clinical oversight, conducts or co-signs assessments, and signs off on treatment. BHP oversight is required and is built into how the facility is paid.

- **Behavioral Health Technicians and Paraprofessionals.** The direct-care staff who deliver day-to-day services and supervision under the BHP's direction, trained and qualified as the rules require.
- **Fingerprint Clearance Cards.** Personnel must hold Arizona Fingerprint Clearance Cards and clear the required background screening before they work with residents.

The BHP is not optional — and not free

You cannot run a BHRF without a Behavioral Health Professional providing clinical oversight, and their oversight is bundled into the AHCCCS per-diem rather than billed separately. Line up your BHP relationship — employed or contracted — before you open, and price their time into your model. A BHRF that cannot demonstrate real BHP oversight will fail both survey and audit.

Part 5 — How Care Works: Admission to Discharge

In Arizona's system a BHRF is a level of care a member is placed into, not a service a person walks in and buys. Understanding the clinical flow is essential, because it is also the billing flow — the money follows the documentation.

- **Referral and authorization.** A member is referred by their outpatient or inpatient treatment team, crisis services, a hospital, or the courts. Admission requires that the member meet medical-necessity criteria and — for AHCCCS members — prior authorization, with continued authorization to keep the stay going.
- **Assessment and treatment plan.** A behavioral health assessment is completed at admission (generally within 48 hours), and a treatment plan is developed with the outpatient treatment team, with BHP sign-off. The plan drives the care and the documentation.
- **The treatment itself.** The BHRF provides the services in the plan — counseling and therapy support, skills building, symptom and medication management support, substance use treatment, structure, and 24-hour supervision — as a bundled level of care.
- **Medication-Assisted Treatment.** A BHRF cannot exclude a member because they are on Medication-Assisted Treatment for opioid use disorder; Arizona law requires that MAT members be admitted and supported.
- **Continued stay and discharge.** The team reassesses progress and risk, justifies continued stay, and plans discharge to a lower level of care — which is the goal of the whole stay.

You are part of a team, not an island

A BHRF does not treat in isolation — it works with the member's outpatient treatment team, which is involved in the treatment plan and the discharge. Services the member needs that your BHRF does not provide (specialty counseling, certain step-down programs) are arranged and authorized separately through that team. Build strong working relationships with the outpatient providers and health plans that refer to you; they are your admission pipeline and your discharge partners.

Where your residents come from

Unlike a private-pay business you market to families, a BHRF fills its beds through the behavioral health system. Knowing who sends you residents — and building trust with them — is how you stay full. Your referral sources are:

- **The AHCCCS Complete Care health plans.** Their care managers place members into residential care with contracted, in-network BHRFs. Being contracted and reliable is what gets you in the rotation.
- **Outpatient behavioral health clinics.** A member's outpatient team refers up to residential when someone needs more than outpatient care, and stays involved through discharge.
- **Crisis services and psychiatric hospitals.** The crisis system and inpatient units need somewhere to step members down to; a BHRF that answers the phone and admits smoothly becomes their go-to.
- **Courts, probation, and justice programs.** Court-ordered and justice-involved placements are a steady source, especially for the secured and recidivism-reduction subclasses.

The through-line is reliability: referral sources place members with the facilities that pick up, admit without drama, communicate, and discharge well. Your reputation with a handful of health-plan care managers and hospital discharge planners is, in practice, your census.

Part 6 — Getting Paid

Nearly every BHRF resident is an AHCCCS (Arizona Medicaid) member, so AHCCCS is your payer. Getting paid means enrolling as an AHCCCS provider, contracting with the health plans, and understanding a per-diem system with one big catch.

Enroll and contract

Register as an AHCCCS provider through the AHCCCS Provider Enrollment Portal (APEP) as the behavioral health residential provider type (Provider Type B8). Then contract with the AHCCCS Complete Care (ACC) health plans — the managed-care organizations that most members are enrolled in — and understand the fee-for-service path for members who are not in a plan. Enrolling with AHCCCS is not the same as contracting with a plan; you generally need both to serve that plan's members and get paid.

The enrollment-then-contract sequence trips up new operators, so be clear on it. Enrolling with AHCCCS through APEP makes you a recognized Medicaid provider; it does not by itself let you serve any given member. Most AHCCCS members are enrolled in one of the AHCCCS Complete Care health plans, and to serve a plan's members and be paid, you contract with that plan and join its network. So the realistic path is: enroll with AHCCCS as Provider Type B8, then approach each ACC plan whose members you want to serve and complete its contracting and credentialing. Members who are fee-for-service (not in a plan) are authorized and paid directly through AHCCCS. Plan for this to take time, and start it in parallel with licensing so your contracts are close to ready when your license issues.

The per-diem — and the room-and-board catch

AHCCCS pays for a BHRF as a per-diem: one bundled daily rate that covers the treatment, supervision, and BHP oversight for a 24-hour day. The BHRF per-diem is billed with HCPCS code H0018 (short-term residential); a long-term residential per-diem, H0019, also exists. The rate is set by AHCCCS and the health plan, and the stay requires prior and continued authorization — for substance use residential, expect a Certification of Need with an ASAM assessment attached. Because the per-diem is a bundle, you generally cannot separately bill the counseling, the BHP's oversight, or the nursing that happen inside the BHRF — those are built into the daily rate. Services a member needs that your BHRF does not provide are arranged and billed by the outpatient team, not added onto your per-diem.

The single most important financial fact

The AHCCCS per-diem does NOT include room and board. The daily rate pays for treatment — not for the bed, the meals, or the roof. Medicaid generally cannot pay for room and board in this setting, so you must plan another way to cover it: the resident's own income or SSI, a room-and-board charge, grant or state funding, or a housing subsidy. Operators who model the per-diem as if it covered housing run out of money. Build your room-and-board plan before you open.

Service (billing) codes

AHCCCS and the member's health plan assign the exact code, modifier, and rate, and residential care is authorized by level. Confirm everything with AHCCCS and your contracted plan before you bill. As a starting reference:

Code	What it is	Notes
H0018	Behavioral health short-term residential, per diem	The core BHRF per-diem; WITHOUT room and board; prior auth required
H0019	Behavioral health long-term residential, per diem	Longer-stay residential per-diem
Room & board	Housing, meals — NOT in the per-diem	Covered by resident income/SSI, a room-and-board charge, or other funding — not Medicaid

Codes are national HCPCS references; AHCCCS and your health plan set the exact code, modifier, rate, and authorization rules. Verify before billing. Claims are submitted on the CMS 1500 (837P) through the AHCCCS portal or the plan.

Finding your actual per-diem rate

This guide does not print a per-diem dollar figure on purpose — because there is no single number that applies to every BHRF, and the right figure for you is one you should confirm, not assume. AHCCCS publishes dated

behavioral health fee schedules on its Rates and Billing pages, in two versions: a fee-for-service (FFS) schedule and a managed-care organization (MCO) schedule. You can find H0018 on the current-dated schedule to see the published rate — but treat the FFS figure as a reference point, because most BHRF residents are in AHCCCS Complete Care plans, and each plan sets the rate it actually pays you under your contract. AHCCCS also states plainly that a listed rate is not a guarantee of payment, and part of behavioral health residential can be reimbursed “By Report” rather than at a fixed amount. To pin down the number your facility will really receive: look up H0018 on the current AHCCCS behavioral health fee schedule, ask each ACC plan you contract with for its BHRF per-diem, and email AHCCCS at FFSRates@azahcccs.gov with rate questions. Build your budget on the rate your contracted plans confirm in writing — not on a figure from an article or a competitor.

Other funding and the payer mix

Source	What it covers	What to know
AHCCCS (ACC plans / FFS)	The BHRF treatment per-diem	Your primary payer; requires enrollment, plan contract, and authorization
Resident income / SSI	Room and board	The most common way residents cover the housing the per-diem excludes
Private pay / commercial insurance	Treatment and/or room and board	Some operators serve private-pay or commercially insured residents alongside AHCCCS
Grants / state & county funding	Room and board, gaps	Behavioral-health and housing funding streams can help cover non-Medicaid costs

Solving the room-and-board gap

Because the room-and-board gap is the defining financial challenge of a BHRF, it deserves a real plan, not a hope. The common approaches, often combined: collect the resident's own income or SSI toward room and board (many residents have a monthly benefit that can be applied); set a modest, disclosed room-and-board charge within what residents can actually pay; pursue behavioral-health housing subsidies and state or county funding that exist precisely to cover the housing Medicaid will not; and, where it fits your model, serve some private-pay or commercially insured residents whose payment can include room and board. Whatever you choose, model it conservatively — assume vacancy, assume some residents cannot pay much, and make sure the treatment per-diem plus your room-and-board plan actually covers your rent, food, and staffing with margin. A BHRF that fills its beds but cannot cover the roof does not survive.

Part 7 — The Building, Fire & Safety

Because residents live at a BHRF around the clock, the physical building carries real weight in licensing. The facility must meet the residential physical-plant standards for your capacity — adequate bedrooms and bathrooms, living and treatment space, and the safety features the codes require — and pass a fire inspection from the local fire department. If you build or remodel, your drawings must be architect-signed and meet the adopted building and fire codes.

Fire and life safety is where residential applications stall

Handle the fire inspection early. The local fire department's inspection — sprinklers or alarms as required, exits, egress, and occupancy for your capacity — is one of the most common reasons a residential license is delayed. Involve the fire marshal and, if you are remodeling, an architect who knows health-care-institution codes, from the very start. And because you are housing vulnerable residents in Arizona, build extreme-heat safety into the building and your emergency plan: reliable cooling, backup planning, and heat-illness awareness are not optional here.

Part 8 — Your Startup Sequence

1. **Months 1-2:** Form the entity, get your EIN, and secure a property that can realistically meet residential physical-plant and fire requirements at your chosen capacity.
2. **Months 1-3:** Recruit your administrator and BHP (and clinical director if 10+ beds); adopt your BHRF policy manual, forms, and clinical documentation; start Fingerprint Clearance Cards.
3. **Months 2-4:** Complete any remodel with architect-signed drawings; pass the fire inspection; submit the ADHS Article 7 application with fee and drawings.
4. **Months 3-5:** Pass the ADHS survey and receive your license. In parallel, enroll with AHCCCS (Provider Type B8) through APEP and begin contracting with the ACC health plans.
5. **Months 5-6:** Finalize your room-and-board plan, build referral relationships with outpatient teams, crisis services, and hospitals, and accept your first authorized admissions.

Part 9 — Common Mistakes to Avoid

- **Modeling the per-diem as if it covered housing.** It does not include room and board — the single most expensive planning error a new BHRF makes.
- **Confusing a BHRF with a sober living home.** They are different licenses, different rules, and different money. A BHRF provides treatment and is a health care institution.
- **Underestimating fire and physical-plant.** Residential life-safety is where applications stall; handle it first, not last.
- **Opening without the AHCCCS contract.** A license without a plan contract is a building with no revenue. Run both tracks together.
- **Thin clinical documentation.** Assessment, treatment plan, BHP sign-off, and continued-stay justification are what AHCCCS pays for; weak documentation means denied and recouped claims.
- **Excluding MAT members.** Arizona law prohibits turning away members on Medication-Assisted Treatment.
- **Choosing 10+ beds without the team.** Crossing the 10-bed line adds a clinical director and heavier requirements; scale into it.

Part 10 — Staying Licensed & Compliant

Once you are open, compliance is continuous. Keep your ADHS license current and pass relicensure inspections; keep every staff member's Fingerprint Clearance Card and qualifications current; maintain real BHP oversight and complete, timely clinical documentation for every resident; keep authorizations current so stays remain covered; report incidents, deaths, and suspected abuse or neglect as the rules require (to Arizona Adult Protective Services

at 1-877-767-2385 for adults, and the Department of Child Safety for minors); and keep your AHCCCS enrollment and plan contracts in good standing. Run a quality program that tracks outcomes, incidents, authorizations, and audit findings and fixes what they show.

The two failure modes that close BHRFs are worth naming plainly. The first is clinical-documentation failure: when the assessment, treatment plan, BHP sign-off, and continued-stay justification are late, thin, or missing, AHCCCS denies and recoups the per-diem, and the revenue evaporates even though the beds were full. The second is the room-and-board squeeze: an operator who never solved the housing gap slowly bleeds cash on rent and food the per-diem was never meant to cover. Both are avoidable, and both are about planning rather than luck — which is the whole point of doing this homework before you open.

The bottom line

A BHRF is a licensed treatment program, not a house with residents. Clear the two gates — the ADHS Article 7 license and the AHCCCS contract — build a real clinical team around a Behavioral Health Professional, plan for the room and board the per-diem does not pay, and run the facility on medical necessity, authorization, and documentation. Do those, and you have a business that fills a serious need and stays full. Skip any of them, and no amount of demand will keep the doors open.

Two Worked Examples

Example 1 — A small mental-health BHRF

You license an 8-bed BHRF (under the 10-bed line, so no clinical director required) for adults with serious mental illness. You employ an administrator, contract with a licensed BHP for clinical oversight, and staff behavioral health technicians around the clock, all fingerprint-cleared. You pass the ADHS survey, enroll with AHCCCS as Provider Type B8, and contract with two AHCCCS Complete Care plans. Referrals come from outpatient clinics and the crisis system; each admission arrives with an assessment, a treatment plan, and prior authorization. AHCCCS pays your H0018 per-diem for treatment; residents' SSI covers room and board. You reassess and justify continued stay, and discharge each resident to outpatient care.

Example 2 — A substance-use residential BHRF

You open a BHRF focused on substance use treatment. Admissions arrive with a Certification of Need and an ASAM assessment supporting the residential level of care, and you cannot turn away members on Medication-Assisted Treatment — you build MAT coordination into your model. Your BHP oversees substance use counseling and the treatment plan; the outpatient team stays involved and handles services outside your scope. You bill the H0018 per-diem with the required authorization, cover room and board through resident income and a modest room-and-board charge, and measure success by stable step-downs to outpatient treatment. Your referral partners are hospitals, crisis facilities, courts, and outpatient SUD providers.

Who to Call & What to Ask

For...	Who to contact
Licensing a BHRF	ADHS Bureau of Residential Facilities Licensing (azdhs.gov). Ask: what does a complete Article 7 initial application require for my capacity, and what are the current fees and timeframes?
The building & fire	Your local fire department / fire marshal, and an architect familiar with health-care-institution codes. Ask: what do I need to pass a residential fire inspection at my capacity?
Fingerprint Clearance Cards	Arizona Department of Public Safety (azdps.gov). Ask: how do my staff apply for Level One cards?
Becoming an AHCCCS provider	AHCCCS Provider Enrollment (APEP), azahcccs.gov . Ask: how do I enroll as Provider Type B8 (behavioral health residential)?
Getting paid / authorization	The AHCCCS Complete Care health plans and AHCCCS Provider Services, 602-417-7670. Ask: how do I contract, and what are your BHRF authorization and billing rules?
Clinical oversight	A licensed Behavioral Health Professional (BHP). Ask: will you provide the clinical oversight, assessments, and treatment-plan sign-off a BHRF requires?
Reporting abuse	Arizona Adult Protective Services — 1-877-767-2385 (adults); Department of Child Safety — 1-888-767-2445 (minors); 911 for emergencies.

Key Terms

- **BHRF:** Behavioral Health Residential Facility — a licensed 24-hour residential behavioral health treatment setting.
- **Article 7:** the ADHS licensing rules for BHRFs (A.A.C. Title 9, Chapter 10, Article 7).
- **Provider Type B8:** the AHCCCS provider type for a behavioral health residential facility.
- **BHP:** Behavioral Health Professional — the licensed clinician who provides oversight, assessment, and treatment sign-off.
- **Per-diem (H0018):** the bundled daily AHCCCS rate for BHRF treatment — which does not include room and board.
- **Room and board:** housing and meals, not covered by the AHCCCS per-diem; covered by resident income, SSI, or other funding.
- **Prior / continued authorization:** AHCCCS approval required before and during a BHRF stay.
- **ASAM / Certification of Need:** the level-of-care assessment and documentation supporting substance use residential admission.
- **MAT:** Medication-Assisted Treatment — members on MAT cannot be excluded from a BHRF.
- **ACC plan:** an AHCCCS Complete Care managed-care health plan you contract with to serve its members.

Core Rules & References

- A.A.C. Title 9, Chapter 10, Article 7 — ADHS licensing of Behavioral Health Residential Facilities (R9-10-701 et seq.)
- A.A.C. R9-10-101 — definitions, including “behavioral health residential facility”
- A.R.S. § 36-422 & A.A.C. R9-10-105/106 — health-care-institution license application and fees
- A.A.C. R9-10-720 — physical plant and fire-inspection requirements for residential facilities
- AHCCCS AMPM Policy 320-V — BHRF service, medical necessity, and per-diem (billed H0018)
- AHCCCS ACOM Policy 414 — prior and continued authorization for residential (Certification of Need / ASAM)
- Arizona Opioid Epidemic Act (SB 1001, 2018) — Medication-Assisted Treatment cannot be a basis for exclusion
- A.R.S. § 46-454 (adults) & § 13-3620 (minors) — mandatory reporting of abuse, neglect, and exploitation

This guide is a general educational resource for prospective Arizona BHRF operators. It is not legal, clinical, tax, or billing advice, and requirements, fees, codes, and contacts change. Verify every step with the agency named — ADHS, AHCCCS and the health plans, the local fire department, and the Department of Public Safety — before you rely on it. Provisions cited reflect Arizona law and AHCCCS policy at the time of writing.